

Weld County Department of Public Health and Environment

Division of Public Health Services · Community Health · Environmental Health

Mobile Health Unit Program · Greeley, Colorado

Advancing health equity through mobile community-based services across Weld County

Mobile Health Unit Data Systems Assessment

Phase 2: Recommendations and Data Dictionary

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Follows from: Phase 1 Data Quality Assessment (July 2026, 61 pages)

Data basis: $N = 85$ events, $N = 314$ survey responses

Report Date: July 2026

Every recommendation traces back to a specific finding from Phase 1. The data dictionary in Section 5 reflects definitions agreed by all three divisions in August 2026.

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1. What This Document Does

Phase 1 confirmed 16 data quality problems across the three MHU divisions. This document turns each one into a clear action: what needs to be done, who does it, how long it takes, and what done looks like.

The 16 actions are grouped under three goals:

- **Goal 1 (R1–R4): Know Who You Serve.** The tracking sheet has no demographic data across all 49 fields. The programme cannot currently describe who its patients are. Four pathways exist and all four should move forward at the same time.
- **Goal 2 (R5–R10): Trust What You Measure.** Six data fields were defined differently across divisions or recorded inconsistently. Several have been resolved through division meetings held in August 2026. Remaining items need action before Phase 3.
- **Goal 3 (R11–R16): Build Systems That Last.** Six infrastructure problems could cause data loss or make the dataset unusable. Most take less than an hour to fix.

Note on Environmental Health. Environmental Health attended only 11 events across both years and just one event in the first five months of 2026. EH data should be treated as descriptive only until participation increases.

2. Effort and Timeline Guide

Table 1: Effort and Timeline Scale

Rating	Staff time	Coordination needed	Typical timeline
Low	One person, under 2 hours	No meeting or approval needed	This week or this month
Medium	One to two people, 2–8 hours	One meeting or one IT request	Within 30 days
High	Multiple people, 8+ hours	Cross-division coordination	4–8 weeks

3. What Phase 1 Found

3.1 Finding 1: We Cannot Say Who We Serve

The tracking sheet has 49 fields. None capture anything about individual patients. This was confirmed across all four interview groups without prompting.

Public Health Services, Phase 1 p. 24: “ I can say when, I can say where, I can say what, I can say how many. I cannot say who we are serving. “

Community Health, Phase 1 p. 24: “ I cannot directly state that we are serving priority populations. I can assume it but I cannot state it. “

Until this is fixed, the programme cannot answer the most basic question a funder or commissioner will ask.

3.2 Finding 2: Engagement Was Being Counted Three Different Ways

Public Health Services, Community Health, and Environmental Health each counted engagements differently. PHS counted individual patient contacts including education. CH counted one per service interaction. EH counted one per person spoken to. Cross-division engagement totals were not comparable.

August 2026 update – resolved: All three divisions agreed in August 2026 on a single definition: one engagement per person. A family of four equals four engagements even if receiving the same service. See updated data dictionary in Section 5.

3.3 Finding 3: Staff Hours Were Not Measured the Same Way

The main tracking sheet tab told EH to record total hours including travel. The PHS and CH tabs said onsite hours only. All three divisions followed their own instructions correctly, producing numbers that could not be compared.

August 2026 update – resolved: All three divisions agreed in August 2026 to adopt EH’s definition: total hours from office departure to return, including travel, pre/post vehicle checks, and setup. The Public Health Services division noted that immunization events will show higher hours due to mobile unit pickup from Fleet. This is intentional and desirable. As the programme director confirmed: if that unit leaves the parking lot, we count it.

3.4 Finding 4: Address Completeness Collapsed in 2026

In 2025, 68.52% of events had a complete address recorded. In 2026 this fell to 22.58%. Four statistical tests confirmed the drop is real ($\chi^2 = 14.856$, $p < 0.001$). The decline is driven by time, not location. Something changed in how addresses are collected between 2025 and 2026, and geographic mapping for 2026 remains limited until the root cause is identified.

3.5 Finding 5: The Tracking Sheet Could Disappear

The master tracking sheet is stored in one person’s personal OneDrive account. When the original owner left the county, the file was nearly lost permanently. Only a locally saved backup prevented the loss of the programme’s entire history.

Environmental Health, written response, Phase 1 p. 56: “ I do hope the data is being archived by the data team somewhere because we have had this issue in the past where people leave and the documents on OneDrive are gone. “

3.6 Finding 6: What Is Working Well

Phase 1 confirmed six things that are going well and should be protected:

- **PHS engagement is genuinely growing.** Median contacts per event rose from 8 in 2025 to 15 in 2026. This is a statistically confirmed real increase ($p = 0.0015$, large effect).
- **STI testing is applied consistently.** The six-test standard is confirmed across all 32 STI events (median = 6.0, CV = 14.0%).
- **The CH engagement drop is not real.** Raw total fell from 692 to 439 only because 2025 had 8 months of data and 2026 had 5. Adjusted to a full year, CH engagement is stable (1,038 vs 1,054). Per-event count rose from 14 to 31.
- **No patient information is stored in the tracking sheet.** All 49 fields are aggregate

counts. No privacy risk in the current setup.

- **Variable definitions were set up before operations began.** This is better than any peer agency surveyed.
- **68.9% of patients could not get this care anywhere else.** The MHU is filling a real gap, not duplicating existing services.

4. Recommendations

4.1 Goal 1: Know Who You Serve (R1–R4)

All four pathways below need to move forward at the same time. Each covers a different division and a different patient population.

4.1.1 R1 – Request EpiTrax Data Pull from Environmental Health

Phase 1 basis: Phase 1 p. 25: Environmental Health staff confirmed that EpiTrax holds about 150 individual data points per child for every blood lead test conducted, including age, address, and ethnicity. A filtered pull by date range is available on request.

Environmental Health, Phase 1 p. 25: “ There are about 150 data points for each individual person. I can pull it for any time frame you want. “

R1 Request EpiTrax data pull from Environmental Health *Low effort*

What to do: Environmental Health to produce a filtered EpiTrax export covering all MHU blood lead test records from April 2025 to present. Minimum fields needed: child age, address, ethnicity, and test result. Selorm to receive and integrate into Phase 3.

Why this matters: Right now EH blood lead data is just a count. EpiTrax would show who is actually being served and whether elevated results are clustering in particular communities.

Responsible: Environmental Health division to produce the pull. Selorm to process and integrate.

Effort and timeline: **Low effort** – This week. One conversation. No approval needed.

Success criteria: EpiTrax file received with age, address, ethnicity, and test result. Confirmed usable for analysis.

4.1.2 R2 – Contact CDC for a DPP Participant Data Referral

Phase 1 basis: Phase 1 p. 51: Community Health confirmed that Diabetes Prevention Programme participant data flows to the CDC under federal reporting requirements, entirely separate from the MHU tracking sheet.

R2 Contact CDC for DPP participant data referral *Medium effort*

What to do: Community Health division contacts the CDC DPP programme to ask whether a data sharing pathway exists for participant demographics linked to MHU events.

Why this matters: A1c screening is the most commonly used MHU service (62.4% of survey respondents). Without DPP data there is no way to describe who those patients are.

Responsible: Community Health division to initiate contact. Data team to advise on format once a pathway is confirmed.

Effort and timeline: **Medium effort** – Within 30 days. One email plus one follow-up.

Success criteria: Confirmed data sharing pathway exists, or formally documented why not and what will substitute.

4.1.3 R3 – Arrange a De-identified Athena Pull for PHS Patients

Phase 1 basis: Phase 1 p. 9: Every PHS patient is entered into Athena under the Mobile Unit service location. The data is filterable by date range and can produce de-identified demographics including age, sex, ethnicity, insurance status, and zip code.

Project inception meeting, Phase 1 p. 9: “ The electronic medical record, Athena, behind this has all of the individual level data. All of the demographics. “

R3 Arrange de-identified Athena pull for PHS patients *High effort*

What to do: Public Health Services clinical staff confirm with the Athena contract holder how MHU events are tagged. The data team defines the extract format. HIPAA data use agreement reviewed before transfer. An introduction between Selorm and the relevant PHS and Athena contacts has already been made by the Public Health Services Director.

Why this matters: PHS delivers the highest volume of clinical services including all STI testing. Without Athena data there is no way to describe who is receiving those services.

Responsible: PHS clinical staff to confirm Athena setup. Data team to coordinate the extract.

Effort and timeline: **High effort** – 4 to 8 weeks. Requires vendor coordination and HIPAA agreement.

Success criteria: De-identified demographic report covering all PHS MHU patients from May 2025 to present, repeatable quarterly.

4.1.4 R4 – Add an Event Population Category Field

Phase 1 basis: Phase 1 p. 20: Event type already encodes useful population information. A food pantry event serves an income-verified population by definition. A Spanish-language event signals language profile. Staff make this inference routinely but nothing captures it in the data.

Community Health, Phase 1 p. 26: “ Because you have to be low income to get food bank services, so we can say that. “

R4 Add Event Population Category field to the tracking sheet *Low effort*

What to do: Selorm drafts a short value list (e.g. income-verified, Spanish-language, general public, employee wellness). Data team adds the field to the tracking sheet and Read Me. All divisions complete it for all 85 existing events and all future events.

Why this matters: This gives the programme a simple way to describe who it is serving at each event without needing individual patient data. It is available for all 85 events right

now at no extra cost.

Responsible: Selorm to draft the field and value list. Data team to add. All division leads to complete retroactively.

Effort and timeline: **Low effort** – Within 30 days. Field addition: 30 minutes. Retroactive completion: 1 to 2 hours.

Success criteria: Field exists with defined values. At least 80% of 2026 events completed.

4.2 Goal 2: Trust What You Measure (R5–R10)

Until these issues are resolved, cross-division comparisons will contain errors that cannot be fixed after the fact.

4.2.1 R5 – Update All Read Me Tabs with the Agreed Staff Hours Standard

Phase 1 basis: Phase 1 p. 39: Two contradictory written instructions existed in the same workbook. Environmental Health recorded total hours including travel. Public Health Services and Community Health recorded onsite hours only.

August 2026 update – resolved: All three divisions agreed in August 2026 to record total hours from office departure to return, including travel, vehicle checks, and setup. All three Read Me tabs must be updated immediately to reflect this change.

R5 Update all three Read Me tabs with the agreed staff hours standard *Low effort*

What to do: Data team updates all three Read Me tabs to read: Record total hours from the time you leave the office to the time you return, including travel, pre/post vehicle checks, and setup and breakdown. Add a change log entry with the date.

Responsible: Data team to update all three tabs. Selorm to confirm all three match.

Effort and timeline: **Low effort** – This week. Under one hour.

Success criteria: All three Read Me tabs contain identical staff hours instructions. Change log entry exists with date and new standard recorded.

4.2.2 R6 – Update All Read Me Tabs with the Agreed Engagement Definition

Phase 1 basis: Phase 1 p. 27: Three divisions counted engagements three different ways, making cross-division totals uninterpretable.

August 2026 update – resolved: All three divisions agreed in August 2026 on one definition: one engagement per person. A family of four at an event equals four engagements regardless of how many receive a service. All three Read Me tabs need to be updated with this definition and a worked example.

R6 Update all three Read Me tabs with the agreed engagement definition *Low effort*

What to do: Data team updates all three Read Me tabs to read: One engagement equals one person. Count every individual present and interacted with, regardless of whether they received a direct service. A family of four equals four engagements. Include this worked example in each tab.

Responsible: Data team to update. Selorm to confirm all three match.

Effort and timeline: **Low effort** – This week. Under one hour.

Success criteria: All three Read Me tabs contain the same engagement definition with the family-of-four worked example.

4.2.3 R7 – Update the CH Read Me to List All Eight Screening Tests

Phase 1 basis: Phase 1 p. 34: The Community Health Read Me listed four screening tests. CH currently offers eight. The data confirmed this independently: the maximum screening ratio observed across all CH events was exactly 8.0.

R7 Update the CH Read Me to list all eight screening tests *Low effort*

What to do: Community Health division updates the Read Me health screenings section to list all eight tests currently offered. Data team confirms the update is live.

Why this matters: Anyone using the Read Me to understand the screening count variable gets the wrong picture if it still says four tests. Takes five minutes to fix.

Responsible: Community Health division to update. Data team to confirm.

Effort and timeline: **Low effort** – This week. Five minutes.

Success criteria: CH Read Me lists all eight tests with date last updated.

4.2.4 R8 – Create an Environmental Health Read Me Tab

Phase 1 basis: Phase 1 p. 39: Environmental Health is the only division without its own Read Me tab. EH data was being interpreted against the main tab instructions, which were not written for EH services or counting conventions.

August 2026 update – resolved: In August 2026 division meetings, all three divisions agreed to adopt EH's three-level attendance field format: Did not attend, Outreach Only, Service and Outreach. The EH Read Me tab should document all confirmed EH definitions including this attendance format, which PHS and CH are now also adopting.

R8 Create an Environmental Health Read Me tab *Medium effort*

What to do: Selorm drafts the EH Read Me content using Phase 1 interview findings and EH written responses. The EH division reviews and confirms accuracy. Data team adds the tab using the same layout as the PHS and CH tabs.

Responsible: Selorm to draft. EH division to review and confirm. Data team to add the tab.

Effort and timeline: **Medium effort** – Within 30 days.

Success criteria: EH Read Me tab exists. EH division has confirmed accuracy. Same layout as other division tabs.

4.2.5 R9 – Add Structured Fields for PHS Services Buried in Notes

Phase 1 basis: Phase 1 p. 10: Narcan distributions, fentanyl test strips, condoms, PrEP navigation conversations, and pregnancy tests appear in the PHS notes field but have no structured column. The 2026 sheet added a Narcan column but only four events have a value despite the notes showing Narcan at more than eight events.

R9 Add structured fields for PHS services currently buried in notes *Medium effort*

What to do: Selorm drafts column names and definitions for all five services. PHS clinical staff confirm the definitions. Data team adds the columns and updates the PHS Read Me. PHS staff complete the new fields retroactively from notes where possible.

Why this matters: Harm reduction services like Narcan are a real and meaningful part of what PHS delivers. If they live only in a free-text notes field they are invisible to any analysis or reporting.

Responsible: Selorm to draft definitions. PHS clinical staff to confirm. Data team to add columns.

Effort and timeline: **Medium effort** – Within 30 days for column creation. Retroactive completion requires additional PHS staff time.

Success criteria: Structured columns exist for all five services. Narcan column being completed consistently. At least 80% of 2026 events filled in retroactively.

4.2.6 R10 – Find Out Why Address Completeness Dropped in 2026

Phase 1 basis: Phase 1 pp. 43–44: Address completeness fell from 68.52% in 2025 to 22.58% in 2026. Four statistical tests confirmed the decline is real and significant ($\chi^2 = 14.856$, $p < 0.001$). The decline is driven by time not location. Something changed in how addresses are collected between 2025 and 2026.

R10 Talk to frontline PHS staff about the address problem *Low effort*

What to do: Selorm conducts two short interviews (15 to 20 minutes each) with frontline Public Health Services staff who regularly fill in the tracking sheet at events. The goal is to find out what changed between 2025 and 2026.

Why this matters: The data confirms something changed but cannot say what. Only the people doing the data entry at events can answer that. Once the cause is known the fix is straightforward.

Responsible: Selorm to conduct the interviews.

Effort and timeline: **Low effort** – Within 30 days.

Success criteria: Root cause identified and documented. Fix implemented before Phase 3. Address completeness in post-fix 2026 events exceeds 50%.

4.3 Goal 3: Build Systems That Last (R11–R16)

4.3.1 R11 – Move the Tracking Sheet Off a Personal Account

Phase 1 basis: Phase 1 pp. 54–56: The master tracking sheet is stored in one staff member’s personal OneDrive account. File ownership has already changed hands once due to staff turnover. The Environmental Health division raised the same concern independently in their written response.

R11 Move the tracking sheet to a shared Data Team account *Low effort*

What to do: The data team creates a shared folder not tied to any individual. IT support needed: approximately 30 minutes. The tracking sheet is moved there and the new location documented in the Read Me.

Why this matters: The programme’s entire two-year operational history lives in this file. It cannot be recovered if it disappears with a departing staff member.

Responsible: Data team to set up the shared location. IT to assist.

Effort and timeline: *Low effort* – This month. Under one hour including IT time.

Success criteria: Tracking sheet in a shared account accessible to the whole data team. New location documented in the Read Me.

4.3.2 R12 – Give Every Event a Unique ID

Phase 1 basis: Phase 1 p. 41: No unique event identifier exists in the tracking sheet. When the same location is visited more than once, Tableau counting functions produce errors. The data team currently works around this by manually editing address strings, which itself introduces inconsistencies.

R12 Assign a unique ID to every event *Low effort*

What to do: Selorm assigns retroactive Event IDs (MHU-2025-001 through MHU-2026-085) to all 85 existing events. Data team adds Event ID as the first column in both tracking sheets. New events get an ID at the time of entry.

Responsible: Selorm to assign the IDs. Data team to update the column structure.

Effort and timeline: *Low effort* – This month. One afternoon.

Success criteria: Every event has a unique ID. New events get an ID at entry. The address-editing workaround is eliminated.

4.3.3 R13 – Set Up a Schema Change Log

Phase 1 basis: Phase 1 p. 41: Between 2025 and 2026, nine new PHS columns were added and two CH columns were renamed without any documentation. It is now impossible to confirm with certainty whether renamed columns measure the same thing as before.

R13 Set up a schema change log *Low effort*

What to do: Selorm writes a one-page protocol as a Read Me addendum. The rule is simple: any column added, renamed, or removed goes in the change log with a date and a reason. Division leads notify the data team before making any field changes.

Responsible: Selorm to write the protocol. Data team to maintain the log going forward.

Effort and timeline: *Low effort* – This month. Under one hour.

Success criteria: Change log exists documenting all known column changes from 2025 to 2026. No new column added without a log entry.

4.3.4 R14 – Add a Quarterly Read Me Review to the Planning Agenda

Phase 1 basis: Phase 1 p. 50: The Read Me documents are the strongest data governance practice the programme has. Both the data team and Community Health independently said a more regular reminder would be useful. Neither had followed through.

R14 Add quarterly Read Me review to MHU planning agenda *Low effort*

What to do: The Public Health Services Director adds a standing agenda item to the quarterly MHU planning meeting: review the Read Me, check whether definitions match current practice, document any gaps.

Why this matters: The engagement definition divergence happened because practice drifted from written definitions over time. A quarterly check is the simplest way to prevent that from happening again.

Responsible: Public Health Services Director to add the agenda item.

Effort and timeline: **Low effort** – This month. Five minutes.

Success criteria: Quarterly meeting includes a standing Read Me review. Gaps documented and resolved within 30 days.

4.3.5 R15 – Check Whether All MHU Events Made It Into the Master Sheet

Phase 1 basis: Phase 1 p. 57: Public Health Services maintains a separate general outreach sheet in addition to the MHU master. Events are transferred manually. If any were missed, the N=85 event count is wrong and every analysis built on it is affected.

R15 Check the PHS outreach sheet against the MHU master *Low effort*

What to do: Selorm requests the PHS outreach sheet from Public Health Services staff and compares it against the MHU master for May 2025 to May 2026. Any discrepancies are investigated and resolved.

Responsible: Selorm to request and compare.

Effort and timeline: **Low effort** – This month. One to two hours.

Success criteria: Both sheets compared. Discrepancies resolved. Phase 3 is built on a confirmed complete dataset.

4.3.6 R16 – Brief Community Health Leadership on the Engagement Numbers Before Any Dashboard Is Shown

Phase 1 basis: Phase 1 pp. 20, 43: Raw CH engagement fell from 692 in 2025 to 439 in 2026. This looks like a 37% drop. It is not. The two years cover different lengths of time (8 months vs 5 months). Adjusted to a full year, CH engagement is stable (1,038 vs 1,054). The per-event count rose from 14 to 31.

R16 Brief Community Health leadership on the engagement numbers *Low effort*

What to do: Selorm and the Public Health Services Director schedule a 30-minute conversation with Community Health leadership before any Phase 3 dashboard is shared. The conversation explains the observation window difference and confirms the correct interpretation.

Why this matters: If Community Health leadership sees a dashboard showing a 37% decline without context they will draw the wrong conclusion about programme performance.

Responsible: Selorm and the Public Health Services Director to schedule.

Effort and timeline: **Low effort** – Before Phase 3 dashboards are shown. One conversation.

Success criteria: Community Health leadership confirms the annualised interpretation. The Phase 3 dashboard shows per-year or per-event counts, never a raw year-over-year total.

5. Data Dictionary

The definitions below reflect what was agreed by all three divisions in meetings held in August 2026. Green text indicates confirmed agreement across all divisions.

Table 2: Agreed Variable Definitions by Division – August 2026

Variable	PHS	CH	EH	Cross-division comparable?
Engagement <i>Agreed Aug 2026</i>	One per person. Family of 4 = 4 engagements even if receiving the same service.	One per person. Changed from per-interaction in August 2026.	One per person spoken to. No change needed – already aligned.	Yes. All three divisions aligned on per person. Read Me tabs to be updated.

Variable	PHS	CH	EH	Cross-division comparable?
People served	Any individual who received a direct clinical service. Education alone does not count.	Any individual who received a health screening. Education alone does not count.	Any individual who received a blood lead test. Education alone does not count.	Yes. Intent consistent. Each division tracks their own services separately. Cross-division duplication is acceptable since each reports to their own grant.
Staff hours <i>Agreed Aug 2026</i>	Total hours from office departure to return, including travel, vehicle checks, and setup. Changed from onsite only.	Total hours from office departure to return. Changed from onsite only. Aligns with payroll since staff are hourly.	Total hours from office departure to return. No change – already the EH standard.	Yes. All three divisions aligned on total hours. PHS immunization events will show higher hours due to Fleet pickup. This is intentional.
Attendance on field <i>Agreed Aug 2026</i>	Three levels: Did not attend / Outreach Only / Service and Outreach. Changed from binary Y/N.	Three levels: Did not attend / Outreach Only / Service and Outreach. Changed from binary Y/N. Note: CH uses Service and Outreach rather than Testing and Outreach.	Three levels: Did not attend / Outreach Only / Service and Outreach. No change – already the EH standard.	Yes. All three divisions aligned on the three-level format.

Variable	PHS	CH	EH	Cross-division comparable?
Health screenings (CH only)	N/A	Count of individual screening results. One person generates multiple results. Eight tests are currently offered. Read Me must be updated to list all 8. See R7.	N/A	Division-specific. Not applicable to PHS or EH.
STI tests (PHS only)	Count of individual tests. Six-test standard confirmed (median = 6.0, CV = 14% across 32 events).	N/A	N/A	Division-specific. Not applicable to CH or EH. Standard confirmed and consistent.
Blood lead tests (EH only)	N/A	N/A	Count of individual tests. Each test generates a mandatory EpiTrax entry.	Division-specific. Not applicable to PHS or CH.
Narcan (PHS only)	Count of kits distributed. Education without a kit does not count as a distribution – it counts as an engagement.	N/A	N/A	Division-specific. Column exists but under-used. See R9.

Blank cells	Blank means the service was not offered at this event. Do not enter zero.	Blank means the service was not offered at this event. Do not enter zero.	Blank means the service was not offered at this event. Do not enter zero.	Yes. Consistent across all three divisions. Critical: blank is not missing data. It means the service was not offered.
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6. External Benchmarking

CALPHO coordinated outreach to comparable agencies between June and July 2026. Four agencies gave substantive responses. One (Genesee County, MI) had just received their unit with no systems yet. Three agencies did not respond. Only three local public health agencies in Colorado operate mobile health units: Weld, Arapahoe, and Jefferson.

Table 3: Peer Agency Benchmarking Responses

Agency	Data systems	Data dictionary	Demographics	Quality assurance
Weld County DPHE	Excel + Athena (PHS) + EpiTrax (EH) + CDC DPP (CH)	Yes – Read Me documents set up before operations began	Not yet in tracking sheet. EpiTrax, Athena, and DPP pathways identified.	Written Read Me definitions; no formal audit process beyond quarterly reminder
Arapahoe County, CO	EHR – paper collected at events, entered later	No	In EHR at patient level	Revamping QA as of July 2026
Barren River District Health, KY	Multiple platforms and spreadsheets	No – survey question prompted new internal thinking	Not tracked	No formal process yet
Black Hawk County, IA	Multiple EHR and grant systems – same for mobile and onsite	No	Within each programme database	Grant-mandated audits

Agency	Data systems	Data dictionary	Demographics	Quality assurance
CDPHE	EHR auto-syncing to state immunisation system. Bills insurance through third party.	No	In EHR. Also tracks SVI, travel distance, repeat patients, and staff feedback.	EHR-embedded validation plus data team review. Most mature of all agencies surveyed.
Genesee County, MI	None – unit just delivered June 2026	N/A	N/A	N/A

6.1 What the Benchmarking Tells Us

1. **Weld’s Read Me documents are unique.** None of the four responding agencies with operational systems has a data dictionary. Weld is ahead of the field.
2. **Every other agency captures demographics through an EHR.** Weld is the only agency trying to capture demographic information outside an electronic health record. The fix requires activating the EpiTrax, Athena, and CDC DPP pathways (R1 to R3), not redesigning the tracking sheet.
3. **CDPHE sets the standard for what Phase 3 dashboards should show.** CDPHE tracks who they serve, SVI scores, travel distance, staff adequacy, and repeat patients. Their rural transformation funds begun June 2026 are worth exploring as a potential Weld funding avenue.
4. **Integrated systems eliminate duplicate data entry.** Agencies that use the same system for mobile and onsite services do not carry the separate data-entry burden that Weld currently does. That is the long-term direction for Weld.
5. **Weld is ahead of every agency surveyed.** Across data dictionaries, demographic tracking, and mobile-unit quality assurance, no peer agency has a more mature setup. Phase 1 and Phase 2 are a contribution to the field.

7. Phase 3 Dashboard Planning

Selorm is building the external Tableau dashboard for Phase 3. This section summarises where things stand and what needs to happen before building can begin.

7.1 Current State

- 85 events across two workbooks (2025: 54 events; 2026: 31 events). Three divisions, one shared Excel workbook.
- Phase 3 data file built: 5 sheets, 76 variables, CDC/ATSDR SVI 2022 for all 83 Weld County census tracts, 314 survey responses cleaned and coded for Tableau.
- SVI: Greeley/Evans mean = 0.72 (MEDIUM-HIGH); Southeast = 0.75 (HIGH). 81.2% of

events in MEDIUM-HIGH or HIGH vulnerability regions.

7.2 Current Data Limitations

- **No individual patient demographics.** EpiTrax, Athena, and CDC DPP pathways identified but not yet activated. Demographic panels will be placeholders until R1 to R3 are complete.
- **Only 22.6% of 2026 events have a complete address.** Geographic mapping limited to 2025 data (68.5% complete).
- **EH data is descriptive only** (1 event in 2026). No statistical conclusions can be drawn.
- **No Event IDs yet.** Tableau functions may produce errors on recurring locations until R12 is done.
- **North Region had zero events in 2026** (five in 2025). Any regional trend panel must flag this.

7.3 What Has to Happen Before Phase 3 Starts

1. **R5 and R6:** Read Me tabs updated with the agreed staff hours and engagement definitions. (Already resolved – just needs documentation this week.)
2. **R11:** Tracking sheet moved to a shared account. (**Low effort**)
3. **R12:** Event IDs assigned to all 85 events. (**Low effort**)
4. **R1:** EpiTrax pull requested from Environmental Health. (**Low effort**)
5. **Meeting with the Executive Director** to confirm what the external dashboard should show. The Public Health Services Director has confirmed this needs to happen before Phase 3 design is finalised.

8. Summary Table

Table 4: Phase 2 Recommendations Summary

#	Recommendation	Goal	Responsible	Effort	Timeline
R1	EpiTrax pull from EH	Know Who You Serve	EH division / Selorm	Low	This week
R2	CDC DPP participant data referral	Know Who You Serve	CH division	Med	30 days
R3	De-identified Athena pull for PHS	Know Who You Serve	PHS / Data team	High	4–8 weeks
R4	Event Population Category field	Know Who You Serve	Selorm / Data team	Low	30 days
R5	Update Read Me: staff hours standard	Trust What You Measure	Data team	Low	This week
R6	Update Read Me: engagement definition	Trust What You Measure	Data team	Low	This week
R7	Update CH Read Me: list all 8 tests	Trust What You Measure	CH division / Data team	Low	This week

#	Recommendation	Goal	Responsible	Effort	Timeline
R8	Create EH Read Me tab	Trust What You Measure	Selorm / EH division	Med	30 days
R9	Add fields for PHS services in notes	Trust What You Measure	PHS / Data team	Med	30 days
R10	Talk to PHS staff about address problem	Trust What You Measure	Selorm	Low	30 days
R11	Move tracking sheet to shared account	Build Systems That Last	Data team / IT	Low	This month
R12	Assign Event IDs to all 85 events	Build Systems That Last	Selorm / Data team	Low	This month
R13	Set up schema change log	Build Systems That Last	Selorm / Data team	Low	This month
R14	Add quarterly Read Me review to agenda	Build Systems That Last	PHS Director	Low	This month
R15	Check PHS outreach sheet vs master	Build Systems That Last	Selorm	Low	This month
R16	Brief CH leadership on engagement numbers	Build Systems That Last	Selorm / PHS Director	Low	Before Phase 3

9. References

1. Federal Committee on Statistical Methodology. (2020). *A Framework for Data Quality*. Office of Management and Budget.
2. Buaka, S. (2026). *MHU Data Systems Assessment: Phase 1 Data Quality Assessment*. Weld County Department of Public Health and Environment. 61 pages. [SAS output: 148 pages, $N = 85$ events, $N = 314$ survey responses.]
3. CALPHO Data Modernization Liaison. (2026, July). Mobile Health Unit Information Gathering: Benchmarking Summary. Colorado Association of Local Public Health Officials (CALPHO).
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6. Centers for Disease Control and Prevention / Agency for Toxic Substances and Disease Registry. (2022). *Social Vulnerability Index – Colorado, Census Tract Level*. FIPS 08123 (Weld County). Retrieved July 2026.